

Intermittent Self-Catheterization (ISC)

Patient Information · Emptying the bladder yourself with a thin catheter

WARWIKI

Intermittent self-catheterization (ISC) is a way to empty your bladder yourself. You gently pass a thin tube (catheter), let the urine drain, and take it out. You do this a few times a day. It is used when the bladder does not empty well on its own. Once you learn it, it takes only a few minutes.

Why It's Done

The bladder may not empty fully after some surgeries, with a **nerve-related bladder problem**, after bladder reconstruction, or because of a blockage. Urine left behind can lead to infections and leaking, and high pressure can harm the kidneys. ISC **empties the bladder** on a schedule.

Why Not a Catheter That Stays In?

- Guidelines prefer ISC to a left-in (indwelling) catheter when you can do it safely.
- **No bag.** Nothing is attached between times.
- You will still need regular check-ups of your bladder and kidneys.

It can feel scary at first. Most people, and caregivers, learn it quickly with a nurse's help. A different catheter or a handling aid can help if your hands are weak.

Tips for Your Body

- **If you have a penis:** pull back the foreskin, if you have one, and clean the tip. Hold the penis straight out from your body as you insert the catheter. When you finish, return the foreskin to its normal place.
- **If you have a vulva:** the urine opening sits just above the vaginal opening, below the clitoris. Sit or lie with your knees apart. A mirror and good light help at first. If no urine comes out, the catheter may be in the vagina. Take it out and try again a little higher with a new catheter.
- **After urethral surgery, or with a known narrowing:** never push past a catch. If the catheter will not pass, stop and call your team. Repeated tries can injure the urethra.

Learn the Terms

ISC

Emptying the bladder yourself with a catheter, then taking it out.

Catheter

A thin, soft tube passed into the bladder to drain urine.

Residual urine

Urine left in the bladder after you urinate.

Neurogenic bladder

A bladder that does not work normally because of a nerve problem.

Clean technique

Washing your hands and the area. It does not need to be sterile.

Indwelling catheter

A catheter left in place with a bag.

Hydrophilic catheter

A catheter with a slippery coating that makes it easier to pass.

Does it hurt? Usually not. Most people feel only mild pressure. Catheters are thin and slippery or come with gel. A little discomfort or spotting is common the first few times. A nurse teaches you step by step.

How to Do It

- 1 **Wash your hands**, get your supplies ready, and clean the opening as you were taught.
- 2 **Gently insert** the catheter until urine flows, then just a little farther. **Never force it.** Stop if you feel strong resistance, pain or new bleeding.
- 3 Let the bladder **drain fully**. Pull the catheter out slowly, pausing if more urine comes.
- 4 Throw away a single-use catheter. Clean a reusable one only if it is labeled for reuse, as taught. Wash your hands.

How Often

- Often **4 to 6 times a day**. Your team sets your schedule.
- Do not skip times. The aim is to keep your bladder from getting too full.
- Record how much you drain if you are asked to.

Good to Know

- Bacteria in the urine are common and are treated only if you have **symptoms**.
- About 1 in 4 people who use ISC get repeated infections. Your team can review your routine and your options.
- A caregiver can be taught if you cannot do it yourself.

Call the office that is treating you if you have:

- A temperature of **100.4 °F (38 °C)** or higher, chills, or side or back pain
- Bleeding that does not clear, or pain that keeps getting worse
- New trouble passing a catheter that used to go in easily
- New leaking between catheterizations

Go to an emergency department if:

- You cannot pass the catheter and your bladder feels full or painful
- You have a spinal cord injury and get a pounding headache, flushing or sweating
- You cannot reach the office and are worried

Three Things to Remember

1. ISC empties a bladder that cannot empty well on its own.
2. Keep to your schedule and use clean technique.
3. Never force the catheter.